

CLINICAL STUDY PROTOCOL

Phase I Dose-Escalation Study

A Phase I Open-Label Dose-Escalation Study of [REDACTED] in Combination with Chemotherapy versus Chemotherapy Alone in Participants with Advanced or Metastatic Gastric/Gastroesophageal Junction Adenocarcinoma with High Claudin 18.2 Expression

Sponsor Protocol Identifier	[REDACTED]
Trial Acronym	[REDACTED]
IND Number	Pending — To be assigned by FDA upon submission
Investigational Product Name	[REDACTED]
Trial Phase	Phase I
Intervention Model	Parallel Group, Dose Escalation (3+3 Design)
Blinding	Open-Label (No Blinding)
Number of Arms	2
Sponsor Name	[REDACTED]
Sponsor Legal Address	[REDACTED] United States
CRO / Clinical Trial Personnel	[REDACTED] — CRO Clinical Trial Lead [REDACTED] [REDACTED] CRO Clinical Trial Personnel
Medical Expert Contact	Available 24 hours/day, 7 days/week. Contact details provided in the Trial Reference Manual.
Original Protocol	Yes
Version Number	Version 1.0
Version Date	28 March 2026
Amendment Identifier	Not Applicable. This protocol has not been amended.
ClinicalTrials.gov Identifier	Pending — To be registered prior to enrolment of first participant

SPONSOR APPROVAL AND SIGNATORY

Name: _____
 Title: _____
 Signature: _____

Date: _____
 Protocol Version: Version 1.0
 Protocol Number: [REDACTED]

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LIST OF ABBREVIATIONS AND ACRONYMS

The following abbreviations and acronyms are used in Clinical Study Protocol [REDACTED]. Abbreviations are listed alphabetically. All abbreviations are defined at first use within each major section of the protocol, and a complete glossary of terms is provided in Section 13 (Appendix: Glossary of Terms and Abbreviations).

Table LOA.1 — List of Abbreviations and Acronyms

Abbreviation / Acronym	Definition
ADCC	Antibody-Dependent Cellular Cytotoxicity
ADA	Anti-Drug Antibody
AE	Adverse Event
ALP	Alkaline Phosphatase
ALT	Alanine Aminotransferase
ANC	Absolute Neutrophil Count
aPTT	Activated Partial Thromboplastin Time
AST	Aspartate Aminotransferase
AUC	Area Under the Concentration-Time Curve
BID	Twice Daily (bis in die)
BUN	Blood Urea Nitrogen
CAPOX	Capecitabine + Oxaliplatin
CBC	Complete Blood Count
CDC	Complement-Dependent Cytotoxicity
CEA	Carcinoembryonic Antigen
CI	Confidence Interval
CLDN18.2	Claudin 18.2
C_{max}	Maximum Serum Concentration
CL	Systemic Clearance
CNS	Central Nervous System
CRF	Case Report Form
CRO	Contract Research Organisation
CT	Computed Tomography
CTCAE	Common Terminology Criteria for Adverse Events
ctDNA	Circulating Tumour DNA
CrCl	Creatinine Clearance
CSP	Clinical Study Protocol
DCR	Disease Control Rate

Abbreviation / Acronym	Definition
DEC	Dose Escalation Committee
DLT	Dose-Limiting Toxicity
dMMR	Mismatch Repair Deficient
DSMB	Data and Safety Monitoring Board
ECG	Electrocardiogram
eCRF	Electronic Case Report Form
ECOG	Eastern Cooperative Oncology Group
EDC	Electronic Data Capture
eGFR	Estimated Glomerular Filtration Rate
ELISA	Enzyme-Linked Immunosorbent Assay
EORTC	European Organisation for Research and Treatment of Cancer
ESMO	European Society for Medical Oncology
FFPE	Formalin-Fixed Paraffin-Embedded
FISH	Fluorescence In Situ Hybridisation
5-FU	5-Fluorouracil
G-CSF	Granulocyte Colony-Stimulating Factor
GCP	Good Clinical Practice
GDPR	General Data Protection Regulation
GEJ	Gastroesophageal Junction
GGT	Gamma-Glutamyl Transferase
HBcAb	Hepatitis B Core Antibody
HBsAg	Hepatitis B Surface Antigen
HBV	Hepatitis B Virus
HCV	Hepatitis C Virus
HER2	Human Epidermal Growth Factor Receptor 2
IB	Investigator's Brochure
ICF	Informed Consent Form
ICH	International Council for Harmonisation
IEC	Independent Ethics Committee
IHC	Immunohistochemistry
IMP	Investigational Medicinal Product
IND	Investigational New Drug

Abbreviation / Acronym	Definition
INR	International Normalised Ratio
IRB	Institutional Review Board
IRR	Infusion-Related Reaction
IV	Intravenous
JGCA	Japanese Gastric Cancer Association
LDH	Lactate Dehydrogenase
MCH	Mean Corpuscular Haemoglobin
MCHC	Mean Corpuscular Haemoglobin Concentration
MCV	Mean Corpuscular Volume
MedDRA	Medical Dictionary for Regulatory Activities
mFOLFOX6	Modified FOLFOX6 (Oxaliplatin + Leucovorin + 5-FU)
mg/m²	Milligrams per Square Metre
MMRM	Mixed-Effects Model for Repeated Measures
MRI	Magnetic Resonance Imaging
MSI-H	Microsatellite Instability-High
MTD	Maximum Tolerated Dose
NAb	Neutralising Antibody
NCCN	National Comprehensive Cancer Network
NCI	National Cancer Institute
NIMP	Non-Investigational Medicinal Product
ORR	Objective Response Rate
OS	Overall Survival
PBMC	Peripheral Blood Mononuclear Cell
PD	Progressive Disease / Pharmacodynamics
PFS	Progression-Free Survival
PK	Pharmacokinetics
PO	Per Os (Oral)
PR	Partial Response
PRO	Patient-Reported Outcome
PS	Performance Status
PT	Preferred Term (MedDRA)
PT/INR	Prothrombin Time / International Normalised Ratio

Abbreviation / Acronym	Definition
Q2W	Every 2 Weeks
Q3W	Every 3 Weeks
Q6W	Every 6 Weeks
QLQ	Quality of Life Questionnaire
QoL	Quality of Life
QTcF	QT Interval Corrected by Fridericia's Formula
RBC	Red Blood Cell
RECIST	Response Evaluation Criteria in Solid Tumours
RP2D	Recommended Phase II Dose
SAE	Serious Adverse Event
SAP	Statistical Analysis Plan
SAS	Safety Analysis Set
SES	Socioeconomic Status
SISH	Silver In Situ Hybridisation
SOC	System Organ Class
SoA	Schedule of Activities
STO22	Stomach Cancer Module 22 (EORTC)
t_{1/2}	Terminal Half-Life
TMB	Tumour Mutational Burden
Tmax	Time to Maximum Concentration
TSH	Thyroid-Stimulating Hormone
ULN	Upper Limit of Normal
Vss	Volume of Distribution at Steady State
WBC	White Blood Cell
WOCBP	Women of Childbearing Potential
β-hCG	Beta Human Chorionic Gonadotropin

Notes:

- Abbreviations for laboratory parameters (e.g. WBC, RBC, ALT, AST) are listed here and additionally defined in Table 12.1 (Clinical Laboratory Assessment Panels).
- Abbreviations specific to regulatory submissions (IND, IMP, NIMP) are defined in the context of this protocol and may have jurisdiction-specific equivalents.
- Drug names and proprietary terms are not included in this list. Refer to Section 6 (Trial Intervention and Concomitant Therapy) for full drug descriptions.
- This list reflects all abbreviations used in Version 1.0 of the protocol dated 28 March 2026.

1. PROTOCOL SUMMARY

Subsections 1.1–1.3 provide the synopsis, objectives/estimands, overall design summary, trial schema, and schedule of activities.

1.1 Protocol Synopsis

The protocol synopsis summarises the key features of the trial. Full details are provided in the main protocol sections below.

Table 1.1 — Protocol Synopsis

Field	Details
Full Protocol Title	A Phase I Open-Label Dose-Escalation Study of [REDACTED] in Combination with Chemotherapy versus Chemotherapy Alone in Participants with Advanced or Metastatic Gastric/GEJ Adenocarcinoma with High Claudin 18.2 Expression
Short Title / Acronym	[REDACTED]
Protocol Number	[REDACTED]
Phase	Phase I
Indication	Advanced or metastatic gastric / gastroesophageal junction (GEJ) adenocarcinoma with high Claudin 18.2 expression
Investigational Intervention	[REDACTED] V Q3W + mFOLFOX6 or CAPOX (Arm A)
Control	mFOLFOX6 or CAPOX chemotherapy alone (Arm B)
Intervention Model	Open-label parallel group dose escalation (3+3 design)
Primary Objective	Evaluate safety and tolerability; determine MTD and RP2D of claudixizunib + chemotherapy
Secondary Objectives	Assess OS, PFS, ORR, PK profile, immunogenicity, and QoL
Primary Endpoints	DLTs in Cycle 1; MTD; RP2D; AE profile (CTCAE v5.0)
Secondary Endpoints	OS, PFS, ORR, DCR, PK parameters, ADA incidence
Population	Adults ≥18 years; advanced/metastatic gastric or GEJ adenocarcinoma; CLDN18.2 high (≥40% tumour cells 2+/3+ IHC); ECOG PS 0–2
Number of Participants	Target: 24–36 participants across up to 6 dose-escalation cohorts (3 per cohort, expandable to 6)
Dose Levels (Arm A)	300, 400, 500, 600, 800 mg/m ² IV [REDACTED] Q3W
Trial Duration	Intervention: until DLT, progression, or withdrawal. Participation: up to 24 months follow-up per participant
Committees	Independent Data and Safety Monitoring Board (DSMB); Dose Escalation Committee (DEC)
Adaptive Design	No
Master Protocol	No
Geographic Scope	Multicentre, multinational (US and EU sites)

ADA = anti-drug antibody; AE = adverse event; CAPOX = capecitabine + oxaliplatin; CLDN18.2 = Claudin 18.2; CTCAE = Common Terminology Criteria for Adverse Events; DCR = disease control rate; DEC = Dose Escalation Committee; DLT = dose-limiting toxicity; DSMB = Data and Safety Monitoring Board; ECOG PS = Eastern Cooperative Oncology Group performance status; GEJ = gastroesophageal junction; IHC = immunohistochemistry; IV = intravenous; mFOLFOX6 = modified FOLFOX6 (oxaliplatin + leucovorin + 5-fluorouracil); mg/m² = milligrams per square metre; MTD = maximum tolerated dose; ORR = objective response rate; OS = overall survival; PFS = progression-free survival; PK = pharmacokinetics; Q3W = every 3 weeks; QoL = quality of life; RP2D = recommended Phase II dose.

1.1.1 Primary and Secondary Objectives and Estimands

The primary objective is to evaluate the safety and tolerability of escalating doses of [REDACTED] in combination with mFOLFOX6 or CAPOX chemotherapy (Lordick et al., 2022; Shitara et al., 2023; Shah et al., 2023) in participants with advanced or metastatic gastric/GEJ adenocarcinoma with high Claudin 18.2 expression, and to establish the maximum tolerated dose (MTD) and recommended Phase II dose (RP2D).

The primary estimand (treatment effect) is the characterisation of the dose-limiting toxicity (DLT) rate in participants with CLDN18.2-positive advanced gastric/GEJ adenocarcinoma treated with [REDACTED] plus chemotherapy, including all safety outcomes regardless of treatment discontinuation during Cycle 1 (treatment policy strategy for all intercurrent events).

The secondary objective is to assess preliminary antitumour activity, including overall survival (OS), progression-free survival (PFS), and objective response rate (ORR), in participants treated with [REDACTED] plus chemotherapy compared to chemotherapy alone. Further details can be found in Section 3.

1.1.2 Overall Design

Key aspects of the trial design are summarised below.

Table 1.2 — Overall Trial Design Summary

Intervention	[REDACTED] (IMP)	Population Type	Patients
Intervention Model	Parallel Group	Population Diagnosis	Gastric/GEJ Adenocarcinoma (CLDN18.2+)
Control Type	Active Comparator	Population Age	Min: 18 years; Max: No upper limit
Control Description	mFOLFOX6 or CAPOX	Site Distribution	Multicentre, Multinational
Assignment Method	Non-randomised (Dose Escalation)	Master Protocol	No
Drug/Device Combination	No	Adaptive Trial Design	No

Number of Arms: 2 (Arm A — Experimental; Arm B — Control)

Trial Blind Schema: Open-label; No Blinding at Site Level

Number of Participants: A target of 24–36 participants will be enrolled across up to 6 dose-escalation cohorts.

Total planned duration of trial intervention per participant: Until dose-limiting toxicity, disease progression, or withdrawal of consent.

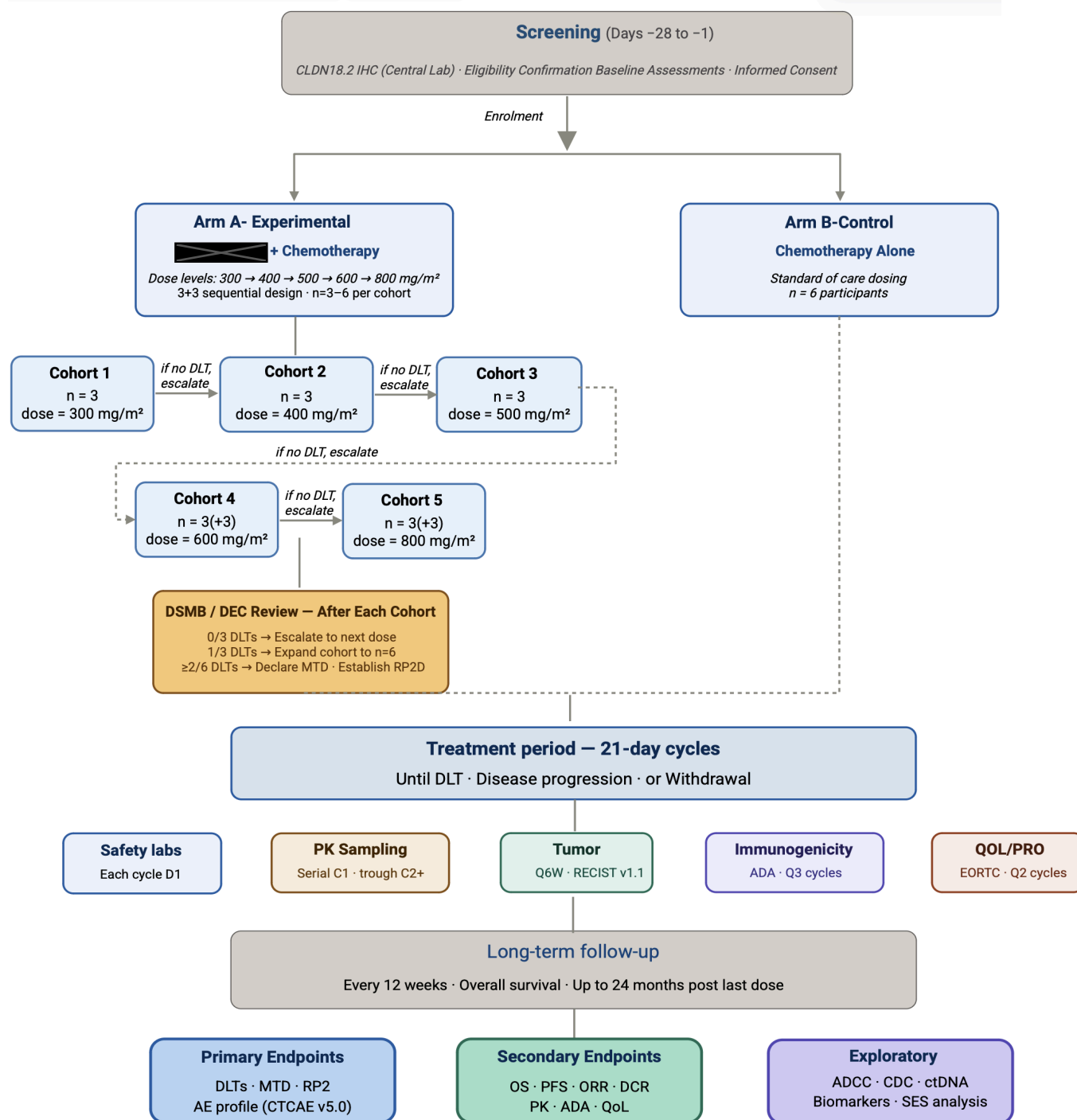
Total planned duration of trial participation per participant: Up to 24 months including follow-up.

Committees: Independent Data and Safety Monitoring Board (DSMB); Dose Escalation Committee (DEC). See Section 11.4.

1.2 Trial Schema

The trial schema below depicts the two arms, participant flow through trial periods, and key milestones including screening, treatment, and follow-up.

Figure 1 — Trial Schema for [REDACTED]: Phase I Dose Escalation Study of [REDACTED]



Abbreviations

ADA = Anti-drug antibody · CAPOX = Capecitabine + Oxaliplatin ·
 DCR = Disease control rate · DEC = Dose Escalation Committee ·
 DLT = Dose-limiting toxicity · DSMB = Data and Safety Monitoring Board ·
 mFOLFOX6 = Modified FOLFOX6 (oxaliplatin + leucovorin + 5-FU) ·
 MTD = Maximum tolerated dose · ORR = Objective response rate ·
 OS = Overall survival · PFS = Progression-free survival ·
 PK = Pharmacokinetics · QoL = Quality of life ·
 RECIST = Response Evaluation Criteria in Solid Tumours ·
 RP2D = Recommended Phase II dose · SES = Socioeconomic status

Chemotherapy = investigator's choice of mFOLFOX6 or CAPOX. Created with BioRender.com

Note: For the 3+3 dose-escalation design, 3 participants are enrolled in each cohort. If 0/3 participants experience a DLT in Cycle 1, escalation proceeds to the next dose level (with DSMB/DEC approval). If 1/3 participants experience a DLT, the cohort expands to 6 participants. If ≥2/6 participants experience DLTs, escalation stops and the previous dose level is declared the MTD.

1.3 Schedule of Activities

Table 1.3 — Schedule of Activities

The schedule of activities captures all procedures at each trial visit. Protocol waivers or exemptions are not permitted. Adherence to this schedule is required.

Procedure	Screening (D-28 to D-1)	C1D1	C1D8	C1D15	Each Cycle D1 (C2+)	Every 2 Cycles (Q6W)	End of Treatment
Informed Consent	X						
I/E Criteria Review	X	X					
Medical History / Demographics	X						
Full Physical Examination	X	X			X		X
Vital Signs	X	X	X	X	X		X
ECOG Performance Status	X	X			X		X
Haematology + Chemistry	X	X	X	X	X		X
Coagulation Profile	X	X			X		X
Urinalysis	X				X		X
12-Lead ECG	X	X					X
Tumour Assessment (CT/MRI)	X					X	X
CLDN18.2 IHC (Central Lab)	X						
PK Sampling (██████████)		X (serial)	X	X	X (trough)		X
Immunogenicity (ADA)	X				X (Q3C)		X
AE / SAE Assessment		X	X	X	X		X
Concomitant Medication Review	X	X	X	X	X		X
EORTC QLQ-C30 / STO22 (QoL)	X					X	X
Pregnancy Test (WOCBP)	X	X			X		X

**Biomarker Sample
(ctDNA, cytokines)**

X	X	X (Q2C)	X
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ADA = anti-drug antibody; C = cycle; D = day; CT = computed tomography; ECOG = Eastern Cooperative Oncology Group; I/E = inclusion/exclusion; IHC = immunohistochemistry; MRI = magnetic resonance imaging; PK = pharmacokinetic; Q2C = every 2 cycles; Q3C = every 3 cycles; Q6W = every 6 weeks; QoL = quality of life; WOCBP = women of childbearing potential.

2. INTRODUCTION

2.1 Purpose of Trial

2.1.1 Trial Rationale

Gastric and GEJ represent a significant unmet medical need with poor prognosis and limited targeted treatment options in the first-line setting. CLDN18.2 has emerged as a clinically validated therapeutic target, with Phase III trials demonstrating that anti-CLDN18.2 antibody therapy significantly improves OS and PFS when combined with standard chemotherapy (Shitara et al., 2023; Shah et al., 2023).

Zolbetuximab (IMAB362) is the first-in-class anti-CLDN18.2 monoclonal antibody to receive regulatory approval for this indication. However, zolbetuximab is a chimeric IgG1 antibody containing murine-derived variable regions, which carries an inherently higher risk of immunogenicity compared to fully humanized antibodies. Furthermore, zolbetuximab's standard IgG1 Fc region provides limited ADCC activity, potentially constraining its antitumour efficacy. Access to zolbetuximab remains limited in many regions globally, and not all patients with CLDN18.2-positive disease achieve durable responses, highlighting a continued unmet need for improved therapeutic options in this population.

[REDACTED] is a humanized IgG1 anti-CLDN18.2 monoclonal antibody developed as a direct competitor to zolbetuximab, specifically engineered to address these limitations. [REDACTED]'s fully humanized framework is expected to reduce the risk of ADA formation compared to zolbetuximab's chimeric structure. [REDACTED] additionally incorporates Fc-region engineering modifications that enhance FcγRIIIa binding affinity, thereby augmenting ADCC and CDC activity beyond that achievable with a standard IgG1 Fc region. The preclinical precedent for this approach is established by published data on Fc-engineered anti-CLDN18.2 antibodies. ZL-1211, a humanized IgG1 anti-CLDN18.2 antibody with Fc-domain mutations enhancing FcγRIIIa binding, demonstrated superior in vitro ADCC activity compared to zolbetuximab across CLDN18.2-high and -low expressing gastric cancer cell lines, with greater antitumour efficacy also observed in mouse xenograft models (Satoh et al., 2022). These preclinical findings support the scientific rationale that Fc-engineering modifications of the type incorporated into [REDACTED] can meaningfully enhance ADCC potency relative to the standard IgG1 Fc region used by zolbetuximab. [REDACTED]-specific preclinical ADCC and immunogenicity data are detailed in the Investigator's Brochure, and collectively provide the scientific rationale for evaluating [REDACTED] in combination with mFOLFOX6 or CAPOX chemotherapy (Lordick et al., 2022; Shitara et al., 2023; Shah et al., 2023) in the same CLDN18.2-positive gastric/GEJ population where zolbetuximab demonstrated clinical benefit.

2.1.2 Background

CLDN18.2 is a splice variant of Claudin 18, a tight junction protein whose expression under normal conditions is confined to differentiated gastric mucosal epithelium, where its epitopes remain inaccessible to systemic antibodies. Upon malignant transformation, disruption of epithelial polarity exposes CLDN18.2 epitopes on the tumour cell surface, making them selectively targetable by systemic antibodies (Sahin et al., 2008). CLDN18.2 is expressed in up to 70–80% of gastric tumours and is aberrantly expressed in pancreatic, oesophageal, and biliary adenocarcinomas, making it one of the most broadly applicable tumour-selective targets in gastrointestinal oncology.

Zolbetuximab (IMAB362), the first-in-class anti-CLDN18.2 chimeric IgG1 monoclonal antibody, established the clinical proof of concept for CLDN18.2-targeted therapy. In its first-in-human Phase I dose-escalation study, zolbetuximab demonstrated acceptable tolerability across doses of 33–1000 mg/m² as a single agent, with no dose-limiting toxicities identified and a recommended Phase II dose range of 300–600 mg/m² (Sahin et al., 2018). Subsequent Phase III evaluation confirmed that zolbetuximab plus mFOLFOX6 (SPOTLIGHT trial) and zolbetuximab plus CAPOX (GLOW trial) each significantly improved PFS and OS compared to chemotherapy alone in CLDN18.2-positive, human epidermal growth factor receptor 2 (HER2)-negative gastric/GEJ cancer — the patient population without an approved targeted therapy option (Shitara et al., 2023; Shah et al., 2023). Despite these

advances, zolbetuximab's chimeric structure and standard IgG1 Fc region represent inherent molecular limitations that [REDACTED] has been specifically engineered to overcome, as described in Section 2.1.1 and the Investigator's Brochure. A complete description of [REDACTED]'s molecular characterisation, preclinical pharmacology, nonclinical safety data, and clinical pharmacology is provided in the Investigator's Brochure.

2.2 Assessment of Risks and Benefits

2.2.1 Risk Summary and Mitigation Strategy

Investigational Intervention: The primary anticipated risks of [REDACTED] are consistent with the established anti-CLDN18.2 drug class safety profile and include gastrointestinal toxicities (nausea, vomiting; on-target effects attributable to CLDN18.2 expression in normal gastric mucosal epithelium), infusion-related reactions (IRRs), and haematologic adverse events related to the chemotherapy backbone. Based on extensive clinical experience with zolbetuximab across Phase I, II, and III trials, no major organ toxicity beyond the gastric mucosa is anticipated, as CLDN18.2 expression in normal tissues is limited to gastric epithelium (Sahin et al., 2018; Shitara et al., 2023; Shah et al., 2023). [REDACTED]'s humanized structure is expected to confer a lower immunogenicity risk than zolbetuximab; however, ADA incidence will be systematically monitored as a secondary safety endpoint given the known class risk. Mandatory antiemetic and antihistamine pre-medication is required before every [REDACTED] infusion as specified in the Investigator's Brochure; specific agents and dosing are provided in the Pharmacy Manual. The DSMB will review cumulative safety data after each cohort before approving dose escalation.

Trial Procedures: Risks associated with IV infusion, phlebotomy, and CT/MRI imaging are standard, well-characterised, and clinically manageable. Optional tumour biopsy carries standard procedural risks commensurate with established oncology practice. Eligibility criteria, dose modification guidelines, and independent DSMB oversight provide systematic risk mitigation throughout the trial.

2.2.2 Benefit Summary

Participants in Arm A may benefit directly from receiving [REDACTED] — a humanized anti-CLDN18.2 antibody with enhanced ADCC activity — in combination with standard-of-care chemotherapy. Based on the clinical benefit demonstrated by the first-in-class anti-CLDN18.2 antibody zolbetuximab in Phase III trials, participants with high CLDN18.2 expression may experience tumour response, prolonged disease control, and OS benefit (Shitara et al., 2023; Shah et al., 2023). [REDACTED]'s molecular differentiation from zolbetuximab — specifically its humanized structure and enhanced Fc-engineering — may provide additional clinical benefit through reduced immunogenicity and superior ADCC-mediated tumour cell killing, though this remains to be established in human studies. All participants across both arms receive established standard-of-care chemotherapy (mFOLFOX6 or CAPOX) consistent with current ESMO and NCCN guidelines (Lordick et al., 2022).

Importantly, Phase I evaluation of [REDACTED] is clinically justified even in the setting of an approved drug in the same class. Zolbetuximab access remains limited in many healthcare systems and not all patients achieve durable responses. [REDACTED]'s differentiated profile may offer benefit to a broader population, including patients who may not respond optimally to zolbetuximab. Participation in this trial also contributes to the scientific knowledge base for CLDN18.2-targeted therapy and supports the development of improved treatments for a cancer with significant unmet need and poor prognosis.

2.2.3 Overall Risk-Benefit Assessment

The risk-benefit profile of [REDACTED] in combination with mFOLFOX6 or CAPOX is considered favourable for Phase I evaluation. The anticipated risks are consistent with those of the established anti-CLDN18.2 drug class and are well-characterised from extensive zolbetuximab clinical data across Phase I, II, and III trials (Sahin et al., 2018; Shitara et al., 2023; Shah et al., 2023). In the zolbetuximab first-in-human Phase I study, 15 patients were treated across five dose levels (33–1000 mg/m²) using a

3+3 design. No dose-limiting toxicities were observed across the full dose range, and no maximum tolerated dose was established. Gastrointestinal toxicities — primarily nausea and vomiting — were the most commonly observed treatment-related adverse events and were consistent with on-target CLDN18.2 expression in normal gastric mucosa. All treatment-related adverse events were manageable, and no treatment-related deaths were reported. A recommended Phase II dose range of 300–600 mg/m² was identified based on pharmacodynamic activity data (Sahin et al., 2018). This well-characterised safety profile across the full dose range, including at 300 mg/m², supports the favourable risk-benefit assessment for ██████████ at the proposed starting dose. ██████████'s humanized structure may confer a more favourable immunogenicity profile than zolbetuximab, potentially reducing ADA-related risks. The mandatory safety monitoring infrastructure including DSMB oversight, dose modification guidelines, antiemetic and antihistamine prophylaxis, and the 21-day DLT observation window — provides robust protection for participants during the highest-risk period of trial conduct. The potential for direct clinical benefit through tumour response and disease control, combined with the scientific importance of establishing the safety and dosing —profile of a molecularly differentiated competitor to an approved drug in a disease with significant unmet need, justifies the Phase I risks for participants with advanced gastric/GEJ adenocarcinoma and high CLDN18.2 expression. More detailed information on the benefit-risk assessment is provided in the Investigator's Brochure.

3. TRIAL OBJECTIVES AND ASSOCIATED ESTIMANDS

3.1 Primary Objective(s) and Associated Estimand(s)

3.1.1 Primary Objective 1

To evaluate the safety and tolerability of escalating doses of [REDACTED] in combination with mFOLFOX6 or CAPOX chemotherapy, and to determine the dose-limiting toxicities (DLTs), maximum tolerated dose (MTD), and recommended Phase II dose (RP2D) in participants with advanced or metastatic CLDN18.2-positive gastric/GEJ adenocarcinoma.

Table 3.1 — Estimand Characteristics — Primary Objective 1 (Safety and Tolerability)

Estimand Characteristic	Description
Population	Adults ≥18 years with histologically confirmed advanced or metastatic gastric/GEJ adenocarcinoma, CLDN18.2 high expression (≥40% tumour cells 2+/3+ IHC), ECOG PS 0–2, treatment-naïve in the metastatic setting
Treatment	[REDACTED] IV Q3W (escalating doses: 300–800 mg/m ²) + mFOLFOX6 or CAPOX chemotherapy (Arm A)
Endpoint	Incidence of DLTs during Cycle 1 (Days 1–21); MTD definition; RP2D determination; all-grade and Grade ≥3 AE profile per CTCAE v5.0
Population-Level Summary	DLT rate (proportion) with exact 95% Clopper-Pearson confidence interval per dose cohort; descriptive summary of all AEs by SOC, preferred term, and CTCAE grade
Intercurrent Events	Treatment policy strategy: all safety events occurring during Cycle 1 are included regardless of treatment discontinuation, dose delay, or dose reduction. Permanent treatment discontinuation due to AE will be captured and analysed.

3.2 Secondary Objective(s) and Associated Estimand(s)

3.2.1 Secondary Objective 1 — Overall Survival

To evaluate overall survival (OS) in participants treated with [REDACTED] plus chemotherapy (Arm A) compared to chemotherapy alone (Arm B).

Table 3.2 — Estimand Characteristics — Secondary Objective 1 (Overall Survival)

Estimand Characteristic	Description
Population	All enrolled participants (Safety Analysis Set)
Treatment	[REDACTED] + chemotherapy (Arm A) vs. chemotherapy alone (Arm B)
Endpoint	Time from first dose to death from any cause
Population-Level Summary	Median OS with 95% CI (Kaplan-Meier); hazard ratio with 95% CI (Cox PH regression)
Intercurrent Events	Treatment policy strategy: OS measured regardless of subsequent anticancer therapy or treatment discontinuation

3.2.2 Secondary Objective 2 — Progression-Free Survival

To evaluate progression-free survival (PFS) per RECIST v1.1 in participants treated with [REDACTED] plus chemotherapy (Arm A) compared to chemotherapy alone (Arm B).

Table 3.3 — Estimand Characteristics — Secondary Objective 2 (Progression-Free Survival)

Estimand Characteristic	Description
Population	Efficacy Analysis Set (participants with ≥ 1 post-baseline tumour assessment)
Treatment	[REDACTED] + chemotherapy (Arm A) vs. chemotherapy alone (Arm B)
Endpoint	Time from first dose to radiological disease progression per RECIST v1.1 or death from any cause, whichever occurs first
Population-Level Summary	Median PFS with 95% CI (Kaplan-Meier); hazard ratio with 95% CI (Cox PH regression)
Intercurrent Events	Treatment policy strategy for discontinuation; censoring rules per RECIST v1.1 guidelines

3.2.3 Secondary Objective 3 — Pharmacokinetics

To characterise the pharmacokinetic (PK) profile of [REDACTED] including C_{max}, T_{max}, AUC, and terminal half-life (t_{1/2}).

3.2.4 Secondary Objective 4 — Immunogenicity

To evaluate the immunogenicity of [REDACTED] as measured by the incidence of anti-drug antibodies (ADA) and neutralising antibodies (NAb).

3.2.5 Secondary Objective 5 — Antitumour Activity

To evaluate objective response rate (ORR) and disease control rate (DCR) per RECIST v1.1, and quality of life (QoL) assessed by EORTC QLQ-C30 and QLQ-STO22.

3.3 Exploratory Objective(s)

3.3.1 Exploratory Objective 1

To evaluate ex vivo pharmacodynamic (PD) biomarkers including ADCC and CDC activity, circulating tumour DNA (ctDNA), and serum cytokine profiles to characterise the pharmacodynamic activity of [REDACTED] and support RP2D determination.

3.3.2 Exploratory Objective 2

To identify predictive biomarkers of response and resistance to [REDACTED], including CLDN18.2 expression level, HER2 status, MSI status, and tumour mutational burden (TMB).

3.3.3 Exploratory Objective 3

To assess the impact of socioeconomic factors on treatment adherence, safety, and clinical outcomes in participants from underprivileged backgrounds.

4. TRIAL DESIGN

4.1 Description of Trial Design

This is an open-label, multicentre, multinational, Phase I dose-escalation trial evaluating [REDACTED] in combination with standard chemotherapy versus chemotherapy alone in participants with advanced or metastatic CLDN18.2-positive gastric/GEJ adenocarcinoma. The trial uses a 3+3 dose-escalation design, which is the established standard approach for Phase I oncology studies allowing systematic safety evaluation at each dose level prior to escalation. Approximately 24–36 participants will be enrolled across up to 6 sequential dose cohorts (300, 400, 500, 600, 800 mg/m² IV [REDACTED] Q3W in Arm A, with a concurrent Arm B chemotherapy control).

A starting dose of 300 mg/m² was selected based on the established safety and PD activity of zolbetuximab at this dose level and preclinical pharmacology data for [REDACTED] (Sahin et al., 2018). Escalation increments of approximately 33% between dose levels (300→400→500→600→800 mg/m²) are consistent with the plateau incremental ratio of the modified Fibonacci sequence, which converges to approximately 1.33-fold increases at higher dose levels and represents the most widely applied dose escalation scheme in Phase I oncology trials (Le Tourneau et al., 2009; Penel et al., 2012). This increment is further supported by the established human safety data for the anti-CLDN18.2 class across doses of 33–1000 mg/m² with no observed DLTs (Sahin et al., 2018), and by preclinical PK/PD modelling for [REDACTED] predicting target serum concentrations in the pharmacodynamically active range at 300 mg/m² (Investigator's Brochure). Given this established safety record, uniform 33% increments across all dose levels are considered appropriate and do not pose undue risk to participants. The 3+3 design does not use formal adaptive randomisation; cohort assignment is sequential. The DSMB and DEC will review all safety data after each cohort completes the 21-day DLT observation window before recommending dose escalation.

4.1.1 Stakeholder Input into Design

This trial design was developed in consultation with the CRO personnel ([REDACTED] and [REDACTED]), oncology investigators with expertise in gastric cancer, and patient advocacy groups representing participants with gastric/GEJ cancer. Specific attention was given to participant burden, the inclusion of participants from underprivileged socioeconomic backgrounds, and consent procedures for participants with limited health literacy.

4.2 Rationale for Trial Design

4.2.1 Rationale for Estimand(s)

The primary estimand uses a treatment policy strategy for all intercurrent events, capturing the full safety impact of [REDACTED] in the population of interest regardless of dose modifications or early discontinuation. This approach is aligned with the primary objective of establishing DLTs and MTD and is the standard approach for Phase I oncology studies. The secondary OS and PFS estimands use a treatment policy strategy consistent with the SPOTLIGHT and GLOW trial designs (Shitara et al., 2023; Shah et al., 2023) to enable cross-trial comparison.

4.2.2 Rationale for Intervention Model

The parallel group design with a concurrent chemotherapy control arm (Arm B) allows direct assessment of the contribution of [REDACTED] to the overall safety and efficacy profile of the combination. Because [REDACTED] is being evaluated as a competitor to zolbetuximab in the same patient population and with the same chemotherapy backbones used in the SPOTLIGHT and GLOW trials (Shitara et al., 2023; Shah et al., 2023), the inclusion of a concurrent control arm also enables preliminary cross-arm OS and PFS comparisons that can be contextualised against the published zolbetuximab Phase III data. This contextualisation will be critical for Phase II/III design decisions and

for regulatory discussions about the clinical differentiation of [REDACTED] from zolbetuximab. The 3+3 dose-escalation model is the most widely used Phase I oncology design and is considered ethically appropriate for systematic safety evaluation in a first-in-combination human study (Le Tourneau et al., 2009).

4.2.3 Rationale for Control Type

mFOLFOX6 and CAPOX are the international standard-of-care regimens for first-line treatment of advanced gastric/GEJ cancer, consistent with NCCN, ESMO, and JGCA guidelines (Lordick et al., 2022; Shitara et al., 2023; Shah et al., 2023). The active comparator control arm provides concurrent safety and preliminary efficacy data essential for Phase I/II benefit-risk characterization.

4.2.4 Rationale for Trial Duration

The 21-day DLT observation window (Cycle 1) is aligned with the Q3W dosing schedule of [REDACTED] and is consistent with Phase I oncology precedent (Le Tourneau et al., 2009; Shitara et al., 2023). The 24-month follow-up period is appropriate to capture mature OS data in a population with median OS of 9–11 months on standard chemotherapy (Iacovelli et al., 2014).

4.2.5 Rationale for Adaptive or Novel Trial Design

Not applicable. This trial does not use an adaptive design.

4.2.6 Rationale for Interim Analysis

No formal interim analysis for efficacy is planned given the exploratory nature of OS/PFS analyses in this Phase I study. The DSMB will conduct ongoing safety reviews after each cohort as part of the dose-escalation process.

4.2.7 Rationale for Other Trial Design Aspects

The use of a central laboratory for CLDN18.2 IHC testing ensures standardised biomarker assessment and eligibility determination. Decentralised elements including home health nurse visits for blood draws and telemedicine check-ins may be implemented to reduce participant burden, particularly for participants from underprivileged backgrounds.

4.3 Trial Stopping Rules

Dose escalation will be halted and no further participants dosed in the next cohort until a full DSMB safety review is completed and dose escalation is approved if:

- Two or more participants in any cohort experience a DLT during Cycle 1
- Any unexpected Grade 4 or 5 treatment-related adverse event occurs
- Any treatment-related death occurs
- A pattern of ≥ 2 Grade 3 treatment-related SAEs of the same preferred term is observed across cohorts

The entire trial will be stopped if the MTD is exceeded at the starting dose level (300 mg/m²) or if the DSMB determines the risk-benefit profile is unacceptable at any dose level.

4.4 Start of Trial and End of Trial

Start of Trial: The trial start date is defined as the date on which the first site is open for participant recruitment (first site initiated).

End of Trial: The end of the trial is defined as the date of the last visit of the last participant in the trial globally. A participant is considered to have completed the trial if the participant has completed all periods of the trial including the final follow-up visit or the last scheduled procedure shown in the SoA.

4.5 Access to Trial Intervention After End of Trial

There is no provision for continued access to [REDACTED] following completion or discontinuation from this Phase I trial outside of a subsequent clinical study. Participants who discontinue will be referred to their treating oncologist for standard-of-care management. Any subsequent Phase II/III clinical trials for [REDACTED] that may open enrolment will be communicated to investigators.

5. TRIAL POPULATION

5.1 Description of Trial Population and Rationale

The trial population consists of adults (≥ 18 years) with histologically confirmed advanced or metastatic gastric or GEJ adenocarcinoma with high CLDN18.2 expression ($\geq 40\%$ of viable tumour cells with 2+ or 3+ membranous staining by validated IHC), who are treatment-naïve in the metastatic setting. This population is selected because: (1) CLDN18.2 high expressors derived the greatest OS and PFS benefit from zolbetuximab plus chemotherapy in Phase III trials (Shah et al., 2023; Shitara et al., 2023); and (2) first-line treatment-naïve participants represent the intended regulatory and clinical use population for [REDACTED].

This trial explicitly includes participants from underprivileged socioeconomic backgrounds, recognising that gastric cancer disproportionately affects lower socioeconomic groups and that these participants have historically been underrepresented in clinical trials. Site selection includes safety-net hospitals and federally qualified health centres. Specific support measures are described in Section 12 (Ethical Considerations for Vulnerable Populations).

Prospective approval of protocol deviations to recruitment and enrollment criteria, also known as protocol waivers or exemptions, is not permitted.

5.2 Inclusion Criteria

To be eligible to participate in this trial, an individual must meet all the following criteria:

1. Participant must be ≥ 18 years of age at the time of signing the informed consent.
2. Histologically or cytologically confirmed advanced (unresectable) or metastatic gastric or gastroesophageal junction (GEJ) adenocarcinoma.
3. High CLDN18.2 expression defined as $\geq 40\%$ of viable tumour cells with moderate to strong (2+ or 3+) membranous CLDN18.2 staining by validated immunohistochemistry (IHC) performed at a central laboratory.
4. Measurable disease per RECIST v1.1 (at least one measurable lesion ≥ 1.0 cm in longest diameter on CT or MRI).
5. Eastern Cooperative Oncology Group (ECOG) performance status of 0, 1, or 2.
6. Estimated life expectancy ≥ 3 months as assessed by the investigator.
7. Adequate organ function within 14 days prior to first dose: (a) ANC $\geq 1.5 \times 10^9/L$; (b) Platelets $\geq 100 \times 10^9/L$; (c) Haemoglobin ≥ 8.0 g/dL; (d) Serum creatinine $\leq 1.5 \times$ ULN or calculated CrCl ≥ 45 mL/min; (e) Total bilirubin $\leq 1.5 \times$ ULN; (f) AST/ALT $\leq 2.5 \times$ ULN ($\leq 5.0 \times$ ULN if liver metastases present).
8. No prior systemic therapy for advanced or metastatic disease (treatment-naïve in metastatic setting). Prior adjuvant/neoadjuvant chemotherapy permitted if completed ≥ 6 months before enrolment.
9. HER2-negative status confirmed by IHC and/or FISH/SISH.
10. Male and/or female participants assigned at birth, inclusive of all gender identities. Contraceptive use by participants or participant partners should be consistent with local regulations. Female participants of childbearing potential must agree to use highly effective contraception during the trial and for ≥ 6 months after last dose. Male participants with partners of childbearing potential must agree to use effective contraception during the trial and for ≥ 3 months after last dose.
11. Signed informed consent as described in Section 11.3, which includes compliance with the requirements and restrictions listed in the ICF and this protocol.

12. Availability of archival tumour tissue (FFPE block or ≥ 5 unstained slides) or willingness to undergo fresh tumour biopsy for central CLDN18.2 testing and biomarker analyses.

5.3 Exclusion Criteria

An individual who meets any of the following criteria will be excluded from participation in this trial:

Medical Conditions:

13. Human epidermal growth factor receptor 2 (HER2)-positive disease, eligible for trastuzumab-based therapy per current standard of care (Bang et al., 2010).
14. Known microsatellite instability-high (MSI-H) or mismatch repair deficient (dMMR) tumours where approved immunotherapy is the recommended standard of care.
15. Active central nervous system (CNS) metastases requiring corticosteroid treatment. Stable treated CNS metastases (≥ 4 weeks post-treatment, off corticosteroids) are permitted.
16. Active autoimmune disease requiring systemic treatment within the past 2 years.
17. Uncontrolled intercurrent illness including uncontrolled hypertension, active infection requiring IV antibiotics, unstable angina, myocardial infarction within 6 months, or symptomatic congestive heart failure.

Liver Safety:

18. Active hepatitis B (HBV DNA >2000 IU/mL) or hepatitis C with active viral replication. Adequately treated chronic hepatitis B or C on suppressive therapy may be enrolled after discussion with the Medical Monitor.

Prior/Concomitant Therapy:

19. Prior treatment with any anti-CLDN18.2 antibody or CLDN18.2-targeting therapy.
20. Prior treatment with mFOLFOX6 or CAPOX in any setting.
21. Known allergy or hypersensitivity to monoclonal antibodies, any component of the [REDACTED] formulation, oxaliplatin, 5-fluorouracil, leucovorin, or capecitabine.
22. Major surgical procedure within 28 days before first dose.

Prior/Concurrent Clinical Trial Experience:

23. Concurrent enrolment in any other investigational drug trial.

Diagnostic Assessments:

24. Prior malignancy within 3 years other than adequately treated non-melanoma skin cancer, in situ cervical cancer, or other localized cancer treated with curative intent.

Other Exclusion Criteria:

25. Pregnancy or breastfeeding.
26. Any condition that in the investigator's judgment would render the participant unable to comply with study procedures or that would constitute an unacceptable safety risk.

5.4 Contraception

5.4.1 Definitions Related to Childbearing Potential

A participant of childbearing potential is defined as any female participant who has not been postmenopausal for at least 12 consecutive months and has not undergone surgical sterilisation (hysterectomy, bilateral oophorectomy, or bilateral salpingectomy). Male participants with partners of childbearing potential are considered to have reproductive potential.

5.4.2 Contraception Requirements

Female participants of childbearing potential must use highly effective contraception (failure rate <1% per year: hormonal contraception, intrauterine device, bilateral tubal ligation, or abstinence from heterosexual intercourse) during the trial and for ≥ 6 months after the last dose of [REDACTED]. Male participants with partners of childbearing potential must use condoms during the trial and for ≥ 3 months after last dose. All participants must agree to avoid sperm or egg donation during the trial.

5.5 Lifestyle Restrictions

5.5.1 Meals and Dietary Restrictions

No specific dietary restrictions are required for [REDACTED] IV infusion. Participants receiving capecitabine (CAPOX arm) must take capecitabine with food as per the prescribing information. Grapefruit or grapefruit juice should be avoided during oxaliplatin-containing chemotherapy.

5.5.2 Caffeine, Alcohol, Tobacco, and Other Restrictions

Participants should be advised to limit alcohol consumption during chemotherapy to reduce hepatotoxicity risk. No restrictions on caffeine, tobacco, or nicotine are mandated by this protocol, though standard medical advice on cessation is encouraged.

5.5.3 Physical Activity Restrictions

No specific physical activity restrictions are mandated. Participants should be advised to avoid strenuous exercise within 24 hours of blood collection for clinical laboratory tests, consistent with standard practice.

5.5.4 Other Activity Restrictions

Participants should refrain from blood or platelet donation during the trial. Participants receiving neurotoxic chemotherapy (oxaliplatin) should be counselled on driving caution if peripheral neuropathy develops.

5.6 Screen Failure and Rescreening

A screen failure occurs when a participant who has consented to participate in the clinical trial is not subsequently assigned to trial intervention or entered in the trial. A minimal set of screen failure information is required to ensure transparent reporting. Minimal information includes demography, screen failure details, eligibility criteria, and any SAE.

Individuals who do not meet the criteria for participation in this trial (screen failure) may be rescreened once, up to 30 days after the initial screening failure, if the reason for screen failure was a transient condition or a laboratory value expected to normalise. A new informed consent is not required for rescreening within this window if the original ICF remains current and valid.

5.7 Criteria for Temporarily Delaying Enrolment/Administration of Trial Intervention

Administration of [REDACTED] must be temporarily delayed if, at the time of scheduled dosing: ANC $< 1.0 \times 10^9/L$; platelets $< 75 \times 10^9/L$; Grade ≥ 3 non-haematologic toxicity unresolved to Grade ≤ 1 ; active uncontrolled infection requiring IV antibiotics. Dosing may resume when the above criteria are met, up to a maximum delay of 14 days within any cycle.

5.8 Recruitment Strategy

Participants will be recruited through oncology outpatient clinics, cancer centres, and safety-net hospitals. Screening for CLDN18.2 expression will be offered broadly to all patients with gastric/GEJ adenocarcinoma at participating sites. Patient navigator programmes will be implemented at all sites to

support recruitment of participants from underprivileged socioeconomic backgrounds (see Section 12). Sites will use ClinicalTrials.gov registration and institutional communication channels for participant identification. Translated recruitment materials will be available in the five most commonly spoken languages at each site.

6. TRIAL INTERVENTION AND CONCOMITANT THERAPY

Trial interventions are all pre-specified, investigational and noninvestigational medicinal products, medical devices, and other interventions intended to be administered to trial participants during the trial conduct.

Table 6.1 — Overview of Trial Interventions

Arm Name	Arm Type	Intervention Name	Type	Dose Form	Dosage Level	Route	IMP/NIMP
Arm A — Experimental	Experimental	██████████ + mFOLFOX6 or CAPOX	Biological + Chemotherapy	Lyophilised powder for IV infusion + oral/IV	300–800 mg/m ² Q3W + standard chemo	IV + IV/PO	IMP (claudixizunib); NIMP (chemo)
Arm B — Control	Active Comparator	mFOLFOX6 or CAPOX	Chemotherapy	IV infusion + oral	Standard institutional dosing	IV + PO	NIMP

6.1 Description of Investigational Trial Intervention

██████████ is a humanized IgG1 monoclonal antibody that selectively binds to the first extracellular domain of CLDN18.2 on the surface of malignant gastric cells. Unlike zolbetuximab, the first-in-class anti-CLDN18.2 antibody which employs a chimeric (murine/human) framework, ██████████ is fully humanized, minimising the risk of anti-drug antibody formation while retaining high binding affinity and selectivity for CLDN18.2. ██████████ mediates antitumour activity through ADCC and CDC mechanisms; its Fc-region engineering modifications enhance FcγRIIIa binding affinity, resulting in superior ADCC potency compared to a standard IgG1 Fc region (Investigator's Brochure). ██████████ is supplied as a sterile lyophilised powder in single-use vials, reconstituted and diluted in 0.9% sodium chloride or 5% dextrose for intravenous (IV) administration. It is administered as a 90-minute IV infusion on Day 1 of each 21-day cycle. Mandatory antiemetic and antihistamine pre-medication must be administered prior to each infusion as specified in the Investigator's Brochure; agents and doses are provided in the Pharmacy Manual.

6.1.1 Medical Devices

Not applicable. No sponsor-provided medical devices are used in this trial.

6.2 Rationale for Investigational Trial Intervention Dose and Regimen

The starting dose of 300 mg/m² Q3W was selected based on three converging lines of evidence, and represents a deliberate and justified departure from the 33 mg/m² starting dose used in the first-in-human zolbetuximab Phase I study (Sahin et al., 2018).

The zolbetuximab Phase I study (Sahin et al., 2018) began at 33 mg/m² because it was a true first-in-human study of an entirely novel drug class with no prior human safety data. That study evaluated five dose levels (33, 100, 300, 600, and 1000 mg/m²) as a single agent in a single-dose design in heavily pretreated patients. No dose-limiting toxicities were observed across the full dose range, confirming broad tolerability of the anti-CLDN18.2 class. The recommended Phase II dose range was established at 300–600 mg/m² based on pharmacodynamic activity data (Sahin et al., 2018). Starting ██████████ at 33 mg/m² in the present trial would therefore be clinically unjustified for four specific reasons.

First, the anti-CLDN18.2 antibody class has an established human safety record across Phase I, II, and III trials involving hundreds of participants (Sahin et al., 2018; Shitara et al., 2023; Shah et al., 2023).

The class safety profile is well characterised and no unexpected major organ toxicities have been identified. Starting at 33 mg/m² — a dose well below established pharmacodynamic activity thresholds — would expose participants to a sub-therapeutic dose without any countervailing safety rationale, which is ethically questionable given that participants have advanced cancer.

Second, this trial evaluates [REDACTED] in a combination regimen with mFOLFOX6 or CAPOX chemotherapy — a meaningfully different context from the single-agent single-dose zolbetuximab Phase I study. The dose-finding question in the present trial is not ‘what is the maximum dose of an entirely unknown drug class?’ but rather ‘what is the optimal dose of a molecularly differentiated drug in a combination setting?’ This refined question is appropriately explored starting from the lowest dose known to have pharmacodynamic activity in humans for the class.

Third, [REDACTED]'s Fc-engineering modifications are predicted to achieve equivalent or superior pharmacodynamic activity at 300 mg/m² compared to zolbetuximab at the same dose, based on preclinical PK/PD modelling predicting target serum concentrations in the EC₅₀ range for ADCC at this dose level (Investigator's Brochure). This further supports 300 mg/m² as a pharmacologically active and scientifically justified starting point.

Fourth, the Q3W dosing schedule used in the present trial differs fundamentally from the single-dose design of the zolbetuximab Phase I study. The 21-day repeated-dosing cycle context — with cumulative drug exposure across multiple cycles — supports selecting a dose level informed by the known pharmacodynamics of the class rather than starting de novo from a minimal dose derived from first-principles toxicology estimates.

The upper dose of 800 mg/m² was selected as the ceiling dose based on the observation that zolbetuximab achieved its maximum pharmacodynamic effect at 600–800 mg/m² in Phase I studies (Sahin et al., 2018), beyond which no additional ADCC or CDC activity was demonstrated. Escalation increments of approximately 33% between dose levels (300→400→500→600→800 mg/m²) follow standard Phase I oncology practice (Le Tourneau et al., 2009), enabling systematic safety characterisation at each dose level before escalation proceeds.

6.3 Investigational Trial Intervention Administration

[REDACTED] is administered IV on Day 1 of each 21-day cycle. Participants must remain in the clinic for a minimum of 2 hours post-infusion for monitoring of infusion-related reactions (IRRs). If a Grade 1 or 2 IRR occurs, the infusion may be interrupted, managed with standard medications, and restarted at a 50% reduced rate. Grade 3 IRRs require permanent discontinuation of [REDACTED].

6.3.1 Retreatment Criteria

Not applicable for this study design (no retreatment period).

6.4 Investigational Trial Intervention Dose Modification

[REDACTED] dose modifications are graded per CTCAE v5.0. Grade 1–2 toxicities attributable to [REDACTED] infusion rate may be reduced by 50%; no dose reduction required. Grade 3 toxicities: interrupt [REDACTED] until recovery to Grade ≤1, then resume at one dose level below. Grade 4 or DLT toxicities: permanent discontinuation of [REDACTED]. A maximum of two dose reductions are permitted per participant. Chemotherapy dose modifications follow institutional protocols for mFOLFOX6 and CAPOX.

6.5 Management of Investigational Trial Intervention Overdose

For this trial, any dose of [REDACTED] greater than 800 mg/m² within a single cycle will be considered an overdose. The sponsor does not recommend a specific antidote. In the event of an overdose, the investigator should evaluate the participant in consultation with the Medical Monitor, closely monitor for AEs, and document the quantity of excess dose and duration.

In the event of an overdose, the investigator should: (a) evaluate whether [REDACTED] should be interrupted or dose reduced; (b) closely monitor the participant for AEs/SAEs; (c) obtain PK samples within 48 hours if requested by the Medical Monitor; and (d) document the quantity and duration of the excess dose.

6.6 Preparation, Storage, Handling and Accountability of Investigational Trial Intervention

6.6.1 Preparation of Investigational Trial Intervention

[REDACTED] lyophilised powder must be reconstituted by a qualified pharmacist or designee using the diluent specified in the Pharmacy Manual. Reconstituted solution must be used within 4 hours at room temperature or 24 hours refrigerated (2–8°C). Preparation instructions are provided in the Pharmacy Manual provided separately to sites.

6.6.2 Storage and Handling of Investigational Trial Intervention

The investigator or designee must confirm appropriate conditions (e.g., temperature) have been maintained during transit for all trial intervention received, and any discrepancies are reported and resolved before use. All trial intervention must be stored in a secure, environmentally controlled, and monitored area in accordance with labelled storage conditions (2–8°C, protected from light) with access limited to the investigator and authorised site staff.

6.6.3 Accountability of Investigational Trial Intervention

Only participants assigned to trial intervention may receive trial intervention, and only authorised site staff may supply, prepare, or administer trial intervention. The investigator is responsible for trial intervention accountability, reconciliation, and record maintenance. Further guidance for final disposition of unused trial interventions is provided in the Trial Reference Manual.

6.7 Investigational Trial Intervention Assignment, Randomization and Blinding

6.7.1 Participant Assignment to Investigational Trial Intervention

Participants are assigned to Arm A ([REDACTED] + chemotherapy) or Arm B (chemotherapy alone) based on cohort sequence in the dose-escalation design. This is not a randomised trial; participant assignment is sequential by cohort. Each participant is assigned a unique participant identification code at enrolment, which is used on all trial documents.

6.7.2 Randomization

Not applicable. This is a non-randomised, open-label trial.

6.7.3 Measures to Maintain Blinding

Not applicable. This is a non-randomised, open-label trial.

6.7.4 Emergency Unblinding at the Site

Not applicable. This is a non-randomised, open-label trial.

6.8 Investigational Trial Intervention Adherence

[REDACTED] is administered by site staff under medical supervision; the date and time of each dose will be recorded in source documents. Chemotherapy adherence will be assessed at each cycle; compliance with oral capecitabine (CAPOX arm) will be assessed by direct questioning and diary review at each visit. Dose delays, reductions, and omissions will be documented.

6.9 Description of Noninvestigational Trial Intervention

6.9.1 Background Trial Intervention

mFOLFOX6 (oxaliplatin 85 mg/m² IV + leucovorin 400 mg/m² IV + 5-FU 400 mg/m² IV bolus + 5-FU 2400 mg/m² over 46h CI, Q2W) or CAPOX (oxaliplatin 130 mg/m² IV + capecitabine 1000 mg/m² PO BID Days 1–14, Q3W) per investigator choice at enrolment.

6.9.2 Rescue Therapy

Antiemetics (ondansetron, metoclopramide, prochlorperazine, or dexamethasone) may be used for nausea and vomiting at any time. Granulocyte colony-stimulating factor (G-CSF) may be used per institutional guidelines for febrile neutropenia or Grade 4 neutropenia. Use of rescue medications must be documented.

6.9.3 Other Noninvestigational Trial Intervention

Not applicable.

6.10 Concomitant Therapy

6.10.1 Prohibited Concomitant Therapy

The following are prohibited during the trial: (a) concurrent enrolment in other investigational drug trials; (b) concurrent systemic anticancer therapy not specified in this protocol; (c) live vaccines within 4 weeks of [REDACTED]; (d) strong CYP3A4 inhibitors or inducers that cannot be substituted, unless approved by the Medical Monitor; (e) other anti-CLDN18.2 directed therapies.

6.10.2 Permitted Concomitant Therapy

Supportive care medications including antiemetics, G-CSF, bisphosphonates, analgesics, antihypertensives, antidiabetics, thyroid hormone replacement, and vitamin/mineral supplements are permitted. Corticosteroids for symptom management are permitted at ≤10 mg/day prednisone equivalent unless for CNS metastases management. All concomitant medications must be recorded.

7. PARTICIPANT DISCONTINUATION OF TRIAL INTERVENTION AND DISCONTINUATION OR WITHDRAWAL FROM TRIAL

7.1 Discontinuation of Trial Intervention for Individual Participants

7.1.1 Permanent Discontinuation of Trial Intervention

[REDACTED] must be permanently discontinued in the following circumstances: (a) Grade 4 treatment-related toxicity; (b) any DLT; (c) Grade 3 IRR; (d) treatment-related SAE deemed incompatible with continued dosing by the investigator or Medical Monitor; (e) radiologically confirmed disease progression per RECIST v1.1 as determined by independent central review, where the investigator determines that continued [REDACTED] administration is no longer in the participant's best interest; this is captured as lack of efficacy and not as an adverse event; (f) participant request; (g) investigator decision in the best interest of participant safety. Chemotherapy may continue per investigator discretion following [REDACTED] discontinuation.

Participants who discontinue [REDACTED] will, where possible, remain in the trial for follow-up assessments to prevent missing data.

7.1.1.1 Liver Event Stopping Criteria

Discontinuation of trial intervention for abnormal liver tests is required by the investigator when a participant meets ALT or AST $\geq 8 \times \text{ULN}$; ALT or AST $\geq 5 \times \text{ULN}$ for more than 2 weeks; ALT or AST $\geq 3 \times \text{ULN}$ with total bilirubin $\geq 2 \times \text{ULN}$ and $>35\%$ direct bilirubin (possible Hy's Law); ALT or AST $\geq 3 \times \text{ULN}$ with symptoms of liver injury. See Appendix 12.4 for further guidance on liver event management.

7.1.1.2 QTc Stopping Criteria

If a clinically significant QTc finding is identified (QTcF >500 ms or increase of >60 ms from baseline) after enrolment, the investigator will determine if the participant can continue on trial intervention and if any change in management is needed. Any new clinically relevant finding should be reported as an AE.

7.1.2 Temporary Discontinuation of Trial Intervention

[REDACTED] may be temporarily interrupted (up to 14 days within a cycle) for: (a) Grade 3 treatment-related non-haematologic AE until recovery to ≤ 1 ; (b) ANC $<1.0 \times 10^9/\text{L}$; (c) platelets $<75 \times 10^9/\text{L}$; (d) active uncontrolled infection. If the delay exceeds 14 days due to toxicity, [REDACTED] should be permanently discontinued. Participants remain in the trial and continue safety assessments during any dose delay.

7.1.3 Rechallenge

Rechallenge with [REDACTED] after a DLT or Grade 4 toxicity is not permitted. Following Grade 3 treatment-related AE (non-DLT), rechallenge at one dose level below may be considered after recovery to ≤ 1 and DSMB/DEC approval. Rechallenge after a liver chemistry stopping criterion is met is not allowed without written sponsor and IRB/IEC approval (see Appendix 12.4).

7.2 Participant Discontinuation or Withdrawal from the Trial

- A participant may withdraw from the trial at any time at the participant's own request for any reason without any negative consequences.
- A participant may be withdrawn at any time at the investigator's discretion for safety, behavioural, or compliance reasons.
- At the time of discontinuing from the trial, an early discontinuation visit should be conducted as shown in the SoA.

- If a participant withdraws consent for future data use, the sponsor may retain and use any data collected prior to withdrawal.
- If a participant withdraws, they may request destruction of untested samples; this must be documented.

7.3 Management of Loss to Follow-Up

A participant will be considered lost to follow-up if the participant repeatedly fails to return for scheduled visits and is unable to be contacted by the trial site. The following actions must be taken: (1) attempt to contact the participant and reschedule the missed visit; (2) before a participant is deemed lost to follow-up, make at least 3 telephone contact attempts and, if necessary, send a certified letter to the last known address; (3) document all contact attempts in the participant's medical record. Site personnel may attempt to collect vital status within legal and ethical boundaries for all participants who are lost to follow-up.

8. TRIAL ASSESSMENTS AND PROCEDURES

- Trial procedures and their timing are summarised in the SoA (Section 1.3). Protocol waivers or exemptions are not allowed.
- Adherence to the trial design requirements, including those specified in the SoA, is essential and required for trial conduct.
- In the event of a significant trial-continuity issue (e.g., pandemic, natural disaster), alternate strategies for participant visits, assessments, and medication distribution may be implemented by the sponsor as per local health authority/ethics requirements.

8.1 Trial Assessments and Procedures Considerations

For all assessments, the same site personnel should perform each assessment throughout the trial whenever possible. Tumour measurements will be performed by centralised independent radiological review. All COA instruments (EORTC QLQ-C30 and QLQ-STO22) should be completed by the participant before any other clinical assessments at each visit to minimise bias. PRO data will be entered electronically via validated ePRO devices or paper as a backup.

8.2 Screening/Baseline Assessments and Procedures

- All screening evaluations must be completed and reviewed to confirm eligibility. The investigator will maintain a screening log.
- Procedures from routine clinical management performed before ICF signing may be used for screening purposes if they meet protocol-specified criteria and were performed within the SoA timeframe.

8.3 Efficacy Assessments and Procedures

Tumour response assessments will be performed by CT of chest/abdomen/pelvis (or MRI when CT is contraindicated) at screening and every 2 cycles (Q6W $\pm$ 7 days) thereafter, using RECIST v1.1 (Eisenhauer et al., 2009), consistent with the imaging schedules used in the SPOTLIGHT and GLOW trials (Shitara et al., 2023; Shah et al., 2023). Independent central radiological review will be used for all tumour assessments contributing to primary and secondary efficacy endpoints. EORTC QLQ-C30 and QLQ-STO22 questionnaires will be completed at baseline and every 2 cycles. Serum tumour markers (CEA, CA19-9, CA72-4) will be collected at baseline and at each cycle Day 1.

8.4 Safety Assessments and Procedures

8.4.1 Physical Examination

A complete physical examination including at minimum assessments of cardiovascular, respiratory, gastrointestinal, neurological, and dermatological systems, plus height/weight measurement, will be performed at screening, Day 1 of each cycle, and at end of treatment.

8.4.2 Vital Signs

Temperature (oral), pulse rate, respiratory rate, and blood pressure will be recorded before blood collection at all scheduled visits. Blood pressure and pulse will be measured after 5 minutes of rest in a seated position. During [REDACTED] infusion, vital signs will be measured pre-dose, at 30 minutes, 60 minutes, at end of infusion, and at 1 and 2 hours post-infusion.

8.4.3 Electrocardiograms

A single 12-lead ECG will be obtained at screening, Day 1 Cycle 1 (pre-[REDACTED] infusion), and at end of treatment. The investigator will review ECG tracings for safety findings. QTcF will be calculated; QTcF >500 ms or increase >60 ms from baseline will be managed per stopping criteria in Section 7.1.1.2.

8.4.4 Clinical Laboratory Assessments

Clinical laboratory tests (haematology, clinical chemistry, coagulation, urinalysis) will be performed at a central laboratory for all protocol-specified assessments, including eligibility confirmation, DLT determination, and scheduled safety monitoring. Central laboratory results are the primary data source for all protocol decisions and eCRF entry. Local laboratory results may be used exclusively for urgent point-of-care participant management decisions where central laboratory turnaround time would result in clinically unacceptable delay — for example, management of acute febrile neutropenia, suspected haemorrhage, or Grade ≥ 3 electrolyte abnormalities requiring immediate intervention. In such cases, a simultaneous central laboratory sample must also be collected and the local result documented in source documents with a clinical justification for its use. Local laboratory results will not be used for DLT determination, dose escalation decisions, or protocol eligibility assessments. Refer to Appendix 12.1 for the complete laboratory panel and Section 11.9 for source data requirements.

8.4.5 Pregnancy Testing

A highly sensitive serum pregnancy test will be performed at screening for all participants of childbearing potential (WOCBP). A repeat serum pregnancy test will be performed on Day 1 of each cycle and at the end of treatment for WOCBP. Additional testing required by local regulations or the IRB/IEC will be performed.

8.4.6 Suicidal Ideation and Behavior Risk Monitoring

[REDACTED] is not a centrally acting or psychoactive intervention and does not have a mechanism of action that would be expected to directly induce or exacerbate suicidal ideation. Formal suicidal ideation and behaviour monitoring instruments (such as the Columbia Suicide Severity Rating Scale, C-SSRS) are therefore not mandated as scheduled assessments in this trial.

However the sponsor recognises that participants with advanced or metastatic gastric cancer carry a significant psychological burden associated with their diagnosis, prognosis, and the demands of trial participation. Clinical investigators and site staff are expected to remain attentive to the psychological wellbeing of participants throughout the trial, consistent with Good Clinical Practice and the principles of the Declaration of Helsinki. If any participant expresses suicidal ideation or psychological distress at any point during trial participation, the investigator must: (1) immediately assess the severity of the situation and refer to appropriate mental health or crisis services; (2) document the event in the source documents; (3) report as an adverse event if clinically significant per CTCAE v5.0 criteria; and (4) notify the sponsor Medical Monitor within 24 hours. Discontinuation from trial intervention may be considered at the investigator's discretion if the participant's psychological status precludes safe continuation.

8.5 Pharmacokinetics

Serial blood samples for PK analysis (4 mL each into serum separator tubes) will be collected during Cycle 1 at the following timepoints: pre-dose, end of infusion (90 min), 3h, 8h, 24h, 48h, 72h, Day 7, Day 14, and Day 21 (= Day 1 of Cycle 2 trough). For Cycles 2 and 3, pre-dose trough samples will be collected. From Cycle 4 onwards, trough samples at Day 1 of each cycle will be collected. At end of treatment, an additional PK sample will be collected. Samples will be processed within 60 minutes and stored at -80°C until batch analysis at a central PK laboratory. [REDACTED] serum concentrations will be determined by validated ELISA. PK parameters (C_{max} , T_{max} , AUClast, AUCinf, $t_{1/2}$, V_{ss} , CL) will be calculated by non-compartmental analysis using Phoenix WinNonlin software.

8.6 Biomarkers

8.6.1 Genetics and Pharmacogenomics

A whole blood sample (8 mL PAXgene tube) for DNA analysis will be collected from consenting participants at screening for exploratory pharmacogenomic analyses. Participation is optional; participants who decline may still enrol in the trial. DNA analyses may include targeted sequencing of genes relevant to [REDACTED] metabolism and gastric cancer biology. Samples will be retained for up to 15 years or until the end of the [REDACTED] development programme, whichever is earlier.

8.6.2 Pharmacodynamic Biomarkers

Blood samples for ex vivo ADCC and CDC pharmacodynamic assays will be collected at Days 1, 14, and 21 of Cycle 1, and at Day 1 of Cycles 3 and 5, from participants in Arm A. PBMCs and serum samples will be analysed for [REDACTED]-mediated ADCC (against CLDN18.2-positive target cells) and CDC activity following methods established for IMAB362 (Sahin et al., 2018). ctDNA samples will be collected at baseline, after Cycle 2, after Cycle 4, and at end of treatment for liquid biopsy analyses.

8.6.3 Other Biomarkers

Serum samples for cytokine profiling (IL-6, IL-8, TNF- α , IFN- γ) will be collected at Cycle 1 Day 1 (pre- and 4h post-infusion) and at Day 1 of Cycle 3. Archival tumour tissue will be analysed for HER2, MSI, TMB, and CLDN18.2 expression level as exploratory predictive biomarkers. In addition to archival tissue, fresh pre-treatment tumour biopsies (within 14 days prior to first dose) and on-treatment biopsies (end of Cycle 2, Day 42 $\pm$ 7 days) are strongly encouraged for all participants where tumour is safely accessible and the procedure is technically feasible. Fresh biopsy samples will be prioritised for pharmacodynamic analyses including CLDN18.2 expression dynamics, immune cell infiltration, and tumour microenvironment characterisation. Participants who are unable or unwilling to undergo fresh biopsy may still enrol in the trial using archival tissue.

8.7 Immunogenicity Assessments

Serum samples for anti-drug antibody (ADA) assessment will be collected at baseline (pre-dose) and at Day 1 of Cycles 2, 4, and 6 (trough), and at end of treatment. ADA samples will be analysed using a validated bridging ELISA assay. Confirmed ADA-positive samples will be further characterised for neutralising antibody (NAb) activity. [REDACTED] serum concentrations will be measured in all ADA samples to enable drug tolerance assessment. Samples may be stored for up to 15 years for further immunogenicity analyses.

8.8 Medical Resource Utilization and Health Economics

Health economics and medical resource utilization parameters are not formal objectives of this Phase I trial. Hospitalisations, emergency room visits, and changes in anticancer treatment due to disease progression or toxicity will be captured as part of the AE/SAE data collection and clinical notes.

9. ADVERSE EVENTS, SERIOUS ADVERSE EVENTS, PRODUCT COMPLAINTS, PREGNANCY AND POSTPARTUM INFORMATION, AND SPECIAL SAFETY SITUATIONS

9.1 Definitions

9.1.1 Definitions of Adverse Events

AE Definition: An AE is any untoward medical occurrence in a clinical trial participant, temporally associated with the use of trial intervention, whether or not considered related to the trial intervention. An AE can therefore be any unfavourable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of trial intervention.

All AEs will be graded using the NCI Common Terminology Criteria for Adverse Events (CTCAE) Version 5.0 and assessed for causality (unrelated, possibly related, probably related, definitely related) by the investigator.

9.1.2 Definitions of Serious Adverse Events

An SAE is defined as any untoward medical occurrence that, at any dose, meets one or more of the following criteria:

- Results in death
- Is life threatening (participant at risk of death at the time of the event)
- Requires inpatient hospitalisation or prolongation of existing hospitalisation
- Results in persistent or significant disability/incapacity
- Is a congenital anomaly/birth defect
- Is a suspected transmission of an infectious agent via an investigational medicinal product
- Is another important medical event that may jeopardise the participant or require intervention to prevent one of the above outcomes

9.1.3 Definitions of Product Complaints

Any complaint about the identity, quality, durability, reliability, safety, or performance of [REDACTED] must be reported to the sponsor within 24 hours of the investigator becoming aware of the complaint. Product complaints include suspected counterfeit or falsified product.

9.1.3.1 Definitions of Medical Device Product Complaints

Not applicable. No sponsor-provided medical devices are used in this trial.

9.2 Timing and Procedures for Collection and Reporting

9.2.1 Timing

All SAEs will be collected from the signing of the ICF until the follow-up visit (30 days post last dose), at the timepoints specified in the SoA.

All AEs will be collected from the signing of the ICF until the follow-up visit (30 days post last dose). Medical occurrences that begin before the start of trial intervention but after obtaining informed consent will be recorded as medical history/current medical conditions.

All SAEs will be recorded and reported to the sponsor or designee immediately and under no circumstances should this exceed 24 hours. The investigator will submit updated SAE data to the sponsor within 24 hours of it being available.

9.2.2 Collection Procedures

The investigator and any qualified designees are responsible for detecting, documenting, and recording events that meet the definition of an AE or SAE. AEs will be identified by open-ended, non-leading questioning at each visit. Severity will be graded per CTCAE v5.0. Causality will be assessed by the investigator for each event. All SAEs will be followed until resolution, stabilisation, or the event is otherwise explained.

9.2.3 Reporting

SAEs will be reported to the sponsor or designee via the electronic data collection (EDC) tool within 24 hours of the investigator's awareness. If the EDC system is unavailable, a paper SAE form will be faxed or emailed to the Medical Monitor. The sponsor has legal responsibility to notify local regulatory authorities and other regulatory agencies about the safety of the investigational intervention under clinical investigation and will comply with country-specific regulatory requirements for safety reporting to regulatory authorities, IRBs/IECs, and investigators.

9.2.4 Adverse Events of Special Interest

The following are designated Adverse Events of Special Interest (AESIs) for this trial, requiring expedited reporting to the sponsor within 24 hours of awareness regardless of seriousness:

- Grade ≥ 3 infusion-related reactions (IRRs) to [REDACTED]
- Grade ≥ 3 gastrointestinal events potentially attributable to CLDN18.2 on-target effects, including but not limited to nausea, vomiting, diarrhoea, haemorrhagic gastritis, gastric perforation, and gastrointestinal haemorrhage
- Any Grade 4 or 5 treatment-related haematologic toxicity
- Any hepatic event meeting liver stopping criteria (see Section 7.1.1.1)
- Any treatment-related QTcF >500 ms or increase >60 ms from baseline

9.2.5 Disease-Related Events or Outcomes Not Qualifying as AEs or SAEs

Disease progression per RECIST v1.1 is an efficacy endpoint and will not be reported as an SAE unless the investigator considers it related to the trial intervention or more severe than expected. Expected disease-related symptoms (pain, dysphagia, weight loss) are recorded as medical history unless they worsen significantly beyond what is expected for the participant's condition.

9.3 Pregnancy and Postpartum Information

9.3.1 Participants Who Become Pregnant During the Trial

Details of all pregnancies in participants of childbearing potential will be collected after the start of trial intervention and until 6 months after the last dose of [REDACTED]. If a pregnancy is reported, the investigator will record pregnancy information and submit it to the sponsor within 24 hours. Any pregnancy complication or elective termination will be reported as an AE or SAE. Abnormal pregnancy outcomes are considered SAEs. Any participant who becomes pregnant while participating in the trial will discontinue [REDACTED]. Chemotherapy will also be discontinued pending review by the investigator and sponsor.

9.3.2 Participants Whose Partners Become Pregnant During the Trial

If the partner of a male participant becomes pregnant during the trial or within 3 months after last dose, the investigator will attempt to collect pregnancy outcome information from the participant (with the partner's consent) and report to the sponsor.

9.4 Special Safety Situations

Special safety situations requiring reporting to the sponsor include: (a) medication errors in preparation or administration of [REDACTED]; (b) suspected product quality issues (e.g., visible particles, colour change); (c) accidental occupational exposure to [REDACTED] by site staff; (d) use of [REDACTED] outside of what is foreseen in the protocol; (e) suspected drug-drug interaction leading to toxicity. All special safety situations must be reported within 24 hours.

10. STATISTICAL CONSIDERATIONS

10.1 General Considerations

A separate Statistical Analysis Plan (SAP) will be developed prior to database lock, detailing all planned analyses. All participant data collected in the trial will be considered in this section. Summary statistics will include mean, standard deviation (SD), median, range, and 95% confidence intervals as appropriate. Categorical variables will be summarised as frequencies and percentages. All analyses will be performed using SAS® Version 9.4 or later. The pre-planned analysis will include all participant data at trial completion.

10.2 Analysis Sets

- Safety Analysis Set (SAS): All participants who receive at least one dose of any trial intervention.
- DLT-Evaluable Set: All participants who complete the DLT observation window (Cycle 1, Days 1–21) or who experience a DLT during Cycle 1.
- Pharmacokinetic Analysis Set (PKAS): All participants who receive [REDACTED] (Arm A) and have at least one evaluable PK sample.
- Efficacy Analysis Set (EAS): All participants in the SAS with at least one post-baseline tumour assessment.

For efficacy analyses, participants will be included according to the intervention they actually received. For safety analyses, all participants who received any dose are included.

10.3 Analyses of Demographics and Other Baseline Variables

Demographic and baseline characteristics will be summarised using descriptive statistics for the SAS by treatment arm and dose cohort. Variables include age, sex, race/ethnicity, ECOG PS, disease stage, primary tumour location, CLDN18.2 expression level, prior therapy, and socioeconomic status indicators (insurance status, area deprivation index). Stratification variables (ECOG PS, geographic region) will be summarised separately.

10.4 Analyses Associated with the Primary Objective(s)

10.4.1 Primary Objective 1

10.4.1.1 Statistical Analysis Method

The DLT rate (proportion of participants experiencing a DLT during Cycle 1) will be calculated for each dose cohort with an exact 95% Clopper-Pearson confidence interval. The MTD will be defined as the highest dose level at which fewer than 2 of 6 participants experience a DLT. All AEs and laboratory abnormalities will be summarised using descriptive statistics by SOC, preferred term (MedDRA), CTCAE grade, and dose cohort. Treatment-related AEs will be summarised separately.

10.4.1.2 Handling of Data in Relation to Primary Estimand(s)

Treatment policy strategy: all DLTs and AEs occurring during Cycle 1 are included regardless of dose modification, treatment interruption, or early discontinuation. Participants who do not complete the DLT observation window for reasons unrelated to toxicity (e.g., rapid disease progression, withdrawal of consent) will be replaced in the DLT-evaluable set and a new participant enrolled in that cohort.

10.4.1.3 Handling of Missing Data in Relation to Primary Estimand(s)

Missing safety data will be handled using a conservative approach: any participant for whom safety data are missing for >50% of the DLT observation window will be replaced as above. No imputation will be performed for laboratory data; missing values will be noted.

10.4.1.4 Sensitivity Analysis

A sensitivity analysis will assess the DLT rate using the modified toxicity probability interval (mTPI) method to confirm dose escalation/de-escalation decisions made by the 3+3 rule.

10.4.1.5 Supplementary Analysis

Dose-response relationships for select safety endpoints (nausea, vomiting, haematologic toxicities) will be explored graphically and using logistic regression models. Further details will be specified in the SAP.

10.5 Analysis Associated with the Secondary Objective(s)

10.5.1 Secondary Objective 1 — Overall Survival

10.5.1.1 Statistical Analysis Method

OS will be estimated using the Kaplan-Meier method. Median OS and 95% confidence intervals will be reported. The log-rank test (stratified by ECOG PS and geographic region) will be used to compare Arms A and B. Hazard ratios and 95% CIs will be estimated using Cox proportional hazards regression. These analyses are exploratory given the Phase I nature and sample size.

10.5.1.2 Handling of Missing Data

Participants lost to follow-up will be censored at the date of last known alive status. All censoring rules follow RECIST v1.1 guidelines for OS.

10.5.2 Secondary Objective 2 — Progression-Free Survival

PFS will be estimated and compared using the same methodology as OS. Censoring will follow standard RECIST v1.1 PFS censoring rules.

10.5.3 Secondary Objectives 3–5

ORR and DCR will be summarised as proportions with exact 95% binomial CIs by treatment arm and dose cohort. PK parameters will be summarised by descriptive statistics by dose cohort; dose proportionality will be assessed by power model analysis. ADA incidence rates will be summarised descriptively. QoL scores (EORTC QLQ-C30 global health status, STO22 subscales) will be summarised as change from baseline using mixed-effects models for repeated measures (MMRM).

10.6 Analyses Associated with the Exploratory Objective(s)

Exploratory biomarker analyses will include: (1) PD/PK correlations for ADCC/CDC activity vs. ██████████ serum concentrations; (2) ctDNA dynamics as a surrogate biomarker of tumour response; (3) association between CLDN18.2 expression level ($\geq 40\%$ vs. $\geq 75\%$ vs. $\geq 90\%$) and clinical outcomes; (4) impact of socioeconomic status on adherence and outcomes. Further details will be provided in the SAP.

10.7 Safety Analyses

All safety data will be summarised using the SAS. AE summaries will include: any AE, Grade ≥ 3 AE, SAE, treatment-related AE, AE leading to discontinuation, AE leading to dose modification. Individual DLT listings will be provided. Laboratory abnormalities will be graded per CTCAE v5.0 and summarised by maximum severity grade. Vital sign and ECG changes from baseline will be summarised descriptively.

10.8 Other Analyses

10.8.1 Other Variables and/or Parameters

Immunogenicity data (ADA incidence, titre, neutralising activity) will be summarised by time point and dose cohort. The relationship between ADA status and PK exposure and clinical outcomes (safety, efficacy) will be explored.

10.8.2 Subgroup Analyses

Pre-specified subgroup analyses for OS and PFS will be conducted by: CLDN18.2 expression level, ECOG PS, geographic region, age (≥ 65 vs. < 65 years), sex, and socioeconomic status. All subgroup analyses are exploratory. Forest plots will be used to display subgroup results. Given the small sample size, these analyses are descriptive only.

10.8.3 Socioeconomic Status Analysis

This trial explicitly enrolls participants from underprivileged socioeconomic backgrounds as part of the equity-focused design described in Sections 3.3.3, 5.1, and 5.8. The following pre-specified exploratory analyses will assess the impact of socioeconomic factors on trial outcomes. All SES analyses are descriptive and hypothesis-generating given the small sample size; no formal statistical testing is planned.

SES Measurement: Socioeconomic status will be assessed using a composite of the following indicators collected at baseline: (1) insurance status (insured vs. underinsured vs. uninsured); (2) Area Deprivation Index (ADI) derived from participant home address, a validated composite measure of neighbourhood socioeconomic disadvantage (Kind et al., 2014); (3) educational attainment (highest level completed); (4) self-reported annual household income bracket; and (5) employment status at enrolment. These variables will be collected via a brief standardised questionnaire administered at screening in the participant's preferred language.

SES Analyses: The following analyses will be conducted:

- Treatment adherence by SES — proportion of planned [REDACTED] doses received, dose delays, and dose reductions will be compared descriptively between participants from lower vs. higher SES groups (defined as ADI quartile 3–4 vs. 1–2).
- Protocol compliance by SES — rate of missed visits, incomplete assessments, and early withdrawal will be compared descriptively between SES groups.
- Safety outcomes by SES — incidence and severity of AEs, rates of hospitalisation, and SAE incidence will be summarised by SES group to assess whether socioeconomic factors are associated with differential toxicity burden.
- Preliminary efficacy signals by SES — OS and PFS will be described by SES group as part of the subgroup analyses in Section 10.8.2, acknowledging the exploratory nature and very small subgroup sizes.
- Support utilisation — use of patient navigator services, translated materials, and financial assistance programmes will be documented and reported descriptively.

These analyses will be reported in the Clinical Study Report and will inform equity-focused design recommendations for subsequent Phase II/III trials of [REDACTED]. The Area Deprivation Index methodology is described in Kind et al. (2014).

10.9 Interim Analyses

No formal interim analysis for efficacy is planned. Safety interim analyses will be performed by the DSMB after each cohort completes the DLT observation window, as described in Section 4.3. DSMB meetings will occur approximately every 3–6 months. The DSMB may recommend dose escalation, cohort expansion, dose de-escalation, or trial termination based on the accumulating safety data.

10.10 Multiplicity Adjustments

This is a Phase I dose-escalation study with a primary safety objective. No formal multiplicity adjustment is required for the primary analysis (DLT rate determination). Secondary OS and PFS analyses are exploratory and hypothesis-generating; type I error control is not mandated. All p-values are nominal and should be interpreted in the context of the Phase I exploratory nature of the trial.

10.11 Sample Size Determination

This trial uses the standard 3+3 dose-escalation design, which does not require a formal sample size calculation for the primary safety objective. The planned sample size of 24–36 participants across up to 6 dose cohorts (3–6 participants per cohort) is consistent with Phase I oncology convention and is sufficient to characterise the DLT rate at each dose level with acceptable precision. The 3+3 design is structured to identify the MTD as the highest dose level at which fewer than 2 of 6 participants experience a DLT, corresponding to a target DLT rate threshold of less than 33% at the RP2D. The statistical operating characteristics of this design are well established: there is a 91% probability of dose escalation if the true DLT rate is 10%, 71% if the true DLT rate is 20%, and less than 50% probability of escalation if the true DLT rate exceeds 30%. Based on the established safety profile of the anti-CLDN18.2 class, a true DLT rate of 10–20% is anticipated at the RP2D, consistent with the observed tolerability of zolbetuximab across Phase I, II, and III trials (Sahin et al., 2018; Shitara et al., 2023; Shah et al., 2023). The sensitivity analysis using the mTPI method (Section 10.4.1.4) will provide confirmatory dose escalation guidance alongside the 3+3 rule. The concurrent Arm B control is sized to provide contextual safety and preliminary efficacy data; no formal power calculation is required.

11. TRIAL OVERSIGHT AND OTHER GENERAL CONSIDERATIONS

11.1 Regulatory and Ethical Considerations

- This trial will be conducted in accordance with the protocol and with: consensus ethical principles derived from international guidelines including the Declaration of Helsinki (WMA, 2013) and CIOMS international ethical guidelines; applicable ICH Good Clinical Practice (GCP) guidelines (ICH E6[R2]); applicable laws and regulations.
- The protocol, protocol amendments, ICF, investigator's brochure, and other relevant documents must be submitted to an IRB/IEC by the investigator and reviewed and approved before implementation.
- Any amendments to the protocol will require IRB/IEC approval before implementation of changes made to the trial design, except for changes necessary to eliminate an immediate hazard to trial participants.
- Protocols and any substantial amendments will require health authority approval prior to initiation except for changes necessary to eliminate an immediate hazard to trial participants.

See Appendix 12.2 for country/region-specific regulatory differences.

11.2 Trial Oversight

11.2.1 Investigator Responsibilities

The investigator is responsible for: conducting the trial in compliance with this protocol, ICH GCP (E6[R2]), applicable regulatory requirements, and IRB/IEC requirements; providing written summaries of trial status to the IRB/IEC annually or as required; notifying the IRB/IEC of SAEs or significant safety findings; ensuring appropriate delegation and oversight of all trial-related duties to qualified personnel; and providing accurate financial disclosure information to the sponsor.

11.2.2 Sponsor Responsibilities

The CRO ([REDACTED]) is responsible on behalf of the sponsor for: oversight of trial conduct; data management and quality assurance; IND/CTA maintenance; regulatory submissions; monitoring; pharmacovigilance reporting; and provision of investigational product.

11.3 Informed Consent Process

- The investigator or representative will explain the nature of the trial, including risks and benefits, and answer all questions regarding the trial.
- Participants must be informed that their participation is voluntary. They will be required to sign a statement of informed consent meeting the requirements of 21 CFR 50, local regulations, ICH guidelines, privacy and data protection requirements, and the IRB/IEC.
- The medical record must include a statement that written informed consent was obtained before enrolment and the date the written consent was obtained.
- Participants must be reconsented to the most current version of the ICF(s) during their participation in the trial.
- A copy of the ICF must be provided to the participant.

For participants with limited literacy or limited English proficiency, consent forms will be available in translated versions at a sixth-grade reading level. Certified medical interpreters will be used; family members are not appropriate interpreters for consent. Patient navigators will assist with the consent process for participants requiring additional support. Verbal consent witnessed by a third party is accepted when written consent is not feasible per applicable IRB guidance.

11.3.1 Informed Consent for Rescreening

A participant who is rescreened within 30 days of the initial screening failure is not required to sign a new ICF if the original ICF remains current. If rescreening occurs after 30 days or if the ICF has been updated, a new ICF must be signed.

11.3.2 Informed Consent for Use of Remaining Samples in Exploratory Research

The ICF will contain a separate section addressing the use of remaining mandatory samples for optional exploratory research. A separate signature will be required. Participants who decline this optional research may still participate in the trial.

11.4 Committees

Independent Data and Safety Monitoring Board (DSMB): An independent DSMB will be established prior to trial initiation and will consist of at least 3 members with expertise in oncology, biostatistics, and clinical trial design. The DSMB will review safety data after each cohort completes the DLT observation window and has authority to recommend dose escalation, de-escalation, or trial termination. A DSMB Charter will define stopping rules, operating procedures, and meeting schedule.

Dose Escalation Committee (DEC): A DEC consisting of the Principal Investigator, Medical Monitor, Sponsor representative, and a biostatistician will review DSMB recommendations and make final dose escalation decisions. All dose escalation decisions will be documented in a Dose Escalation Decision Log.

11.5 Insurance and Indemnity

The sponsor maintains clinical trial insurance covering all enrolled participants for trial-related injury in accordance with applicable local regulatory requirements. Details are provided in the site agreement and will be communicated to participants in the ICF.

11.6 Risk-Based Quality Management

Critical to quality factors identified for this trial include: CLDN18.2 IHC eligibility determination; DLT assessment and reporting; [REDACTED] infusion administration and IRR management; PK sample collection and processing; and OS/PFS tumour assessment accuracy. Risk mitigation strategies include central laboratory testing, mandatory site training, standardised procedures manuals, and centralised monitoring with remote data review. Full details are provided in the Risk Management Plan.

11.7 Data Governance

Records and documents, including signed ICFs, pertaining to the conduct of this trial must be retained by the investigator for 15 years after trial completion unless local regulations or institutional policies require a longer retention period. No records may be destroyed during the retention period without written approval from the sponsor.

11.8 Data Protection

- Participants will be assigned a unique identifier by the sponsor. Any participant records transferred to the sponsor will contain the identifier only; names or identifying information will not be transferred.
- The participant must be informed that their personal trial-related data will be used by the sponsor in accordance with local data protection law, including HIPAA (US) and GDPR (EU).
- The participant must be informed that their medical records may be examined by Clinical Quality Assurance auditors or inspectors from regulatory authorities.

11.9 Source Data

11.9.1 Data Quality Assurance

- All participant data relating to the trial will be recorded on electronic CRFs (eCRFs) in the validated EDC system. The investigator is responsible for verifying data accuracy by electronically signing the eCRF.
- The investigator must permit trial-related monitoring, audits, IRB/IEC review, and regulatory agency inspections and provide direct access to source documents.
- The sponsor or designee is responsible for data management of this trial, including quality checking.

11.9.2 Source Documents

- Source documents provide evidence for the existence of the participant and substantiate the integrity of the data collected. Source documents are filed at the investigator's site.
- Data reported on the eCRF that are transcribed from source documents must be consistent with the source documents or discrepancies must be explained.

11.10 Protocol Deviations

All protocol deviations will be identified, documented, and assessed for their impact on participant safety and data integrity. Deviations will be classified as major or minor per sponsor definitions. All major deviations will be reported to the IRB/IEC per local requirements and to the sponsor within 5 business days of identification. A deviation log will be maintained at each site.

11.11 Early Site Closure

The sponsor or designee reserves the right to close a trial site or terminate the trial at any time for any reason at the sole discretion of the sponsor. Trial sites will be closed upon trial completion. Reasons for early closure include: failure to comply with protocol or GCP requirements; inadequate recruitment; safety concerns; or discontinuation of [REDACTED] development. If the trial is prematurely terminated or suspended, the sponsor will promptly inform investigators, IECs/IRBs, and regulatory authorities. The investigator will inform participants and ensure appropriate therapy and follow-up.

11.12 Data Dissemination

This trial will be registered on ClinicalTrials.gov prior to enrolment of the first participant. Results will be posted on ClinicalTrials.gov within 12 months of study completion or discontinuation, in accordance with applicable regulatory requirements and sponsor policy.

11.12.1 Publication Policy

- The results of this trial may be published or presented at scientific meetings. The investigator agrees to submit all manuscripts or abstracts to the sponsor before submission to allow protection of proprietary information and to provide comments.
- The sponsor will comply with the requirements for publication of trial results. Authorship will be determined by mutual agreement and in line with ICMJE authorship requirements.

12. APPENDIX: SUPPORTING DETAILS

12.1 Clinical Laboratory Tests

All laboratory tests will be performed by a central laboratory unless specified otherwise. Refer to the Laboratory Manual provided separately for sample collection, processing, and shipping instructions.

Panel	Parameters
Haematology	CBC with differential: WBC with differential (neutrophils, lymphocytes, monocytes, eosinophils, basophils), RBC count, haemoglobin, haematocrit, MCV, MCH, MCHC, platelets, reticulocyte count
Clinical Chemistry	Sodium, potassium, chloride, bicarbonate, BUN/urea, creatinine, eGFR, glucose, calcium, magnesium, phosphate, total protein, albumin, total and direct bilirubin, AST, ALT, ALP, GGT, LDH, uric acid
Coagulation	PT/INR, aPTT
Urinalysis	Specific gravity, pH, glucose, protein, blood, ketones by dipstick; microscopy if blood or protein abnormal
Serology / Other	Hepatitis B surface antigen (HBsAg), hepatitis B core antibody (HBcAb), hepatitis C antibody (HCV Ab), HIV antibody (at screening only); TSH (screening); CEA, CA19-9, CA72-4 (screening and Day 1 of each cycle)
Pregnancy Testing	Highly sensitive serum β -hCG (WOCBP only): at screening, Day 1 of each cycle, and at end of treatment

Investigators must document their review of each laboratory safety report. Laboratory values considered clinically significantly abnormal during participation or within 30 days after the last dose should be repeated until values return to normal or baseline, or are no longer considered clinically significant.

12.2 Country/Region-Specific Differences

Where country-specific differences in regulatory requirements cannot be reconciled by flexible protocol language, they will be addressed by country-specific protocol amendments or addenda. Known anticipated differences include: hepatitis B and C testing requirements (Japan, South Korea); contraception requirements (Australia); data retention periods (EU under GDPR). The following countries/regions will participate in this trial: United States, Germany, France, Italy, Japan (planned). No formal enrollment caps by country or region are stipulated in this protocol. The sponsor will monitor geographic distribution of enrollment throughout the trial to ensure the safety and pharmacokinetic dataset is sufficiently diverse to support regulatory submissions across all participating regions. Country-specific addenda will be issued as required prior to site initiation in each country.

12.3 Prior Protocol Amendment(s)

Not applicable. This protocol has not been amended.

Table 12.2 — Protocol Amendment History

Document	Sponsor Approval Date	Approximate Enrollment at Amendment
Original Protocol — Version 1.0	28 March 2026	0 (Pre-initiation)

12.4 Liver Safety: Suggestions and Guidelines for Liver Events

Liver event stopping criteria are defined in Section 7.1.1.1. In the event that a participant meets liver chemistry stopping criteria, the following actions are required: (1) immediately discontinue [REDACTED] and any potentially hepatotoxic chemotherapy; (2) perform repeat liver chemistries within 48 hours; (3) notify the Medical Monitor within 24 hours; (4) consult gastroenterology/hepatology as clinically appropriate; (5) collect a serum sample for hepatitis B and C viral load if not recently tested; (6) consider liver imaging (ultrasound, CT) if clinically indicated. The participant may continue in the trial for follow-up assessments but may not be rechallenged with [REDACTED] without written sponsor and IRB/IEC approval.

13. APPENDIX: GLOSSARY OF TERMS AND ABBREVIATIONS

Table 13.1 — Glossary of Terms and Abbreviations

Abbreviation	Definition
ADCC	Antibody-Dependent Cellular Cytotoxicity
ADA	Anti-Drug Antibody
AE	Adverse Event
ALP	Alkaline Phosphatase
ALT	Alanine Aminotransferase
ANC	Absolute Neutrophil Count
AUC	Area Under the Concentration-Time Curve
CAPOX	Capecitabine + Oxaliplatin
CBC	Complete Blood Count
CDC	Complement-Dependent Cytotoxicity
CI	Confidence Interval
CLDN18.2	Claudin 18.2
CRO	Contract Research Organisation
CT	Computed Tomography
CTCAE	Common Terminology Criteria for Adverse Events
DCR	Disease Control Rate
DEC	Dose Escalation Committee
DLT	Dose-Limiting Toxicity
DSMB	Data and Safety Monitoring Board
ECG	Electrocardiogram
ECOG	Eastern Cooperative Oncology Group
EDC	Electronic Data Capture
eGFR	Estimated Glomerular Filtration Rate
EORTC	European Organisation for Research and Treatment of Cancer
ESMO	European Society for Medical Oncology
FFPE	Formalin-Fixed Paraffin-Embedded
GCP	Good Clinical Practice
GEJ	Gastroesophageal Junction
G-CSF	Granulocyte Colony-Stimulating Factor
ICF	Informed Consent Form
ICH	International Council for Harmonisation

IHC	Immunohistochemistry
IMP	Investigational Medicinal Product
IND	Investigational New Drug
IRB/IEC	Institutional Review Board / Independent Ethics Committee
IRR	Infusion-Related Reaction
IV	Intravenous
JGCA	Japanese Gastric Cancer Association
MedDRA	Medical Dictionary for Regulatory Activities
mFOLFOX6	Modified FOLFOX6 (Oxaliplatin + Leucovorin + 5-FU)
MMRM	Mixed-Effects Model for Repeated Measures
MSI-H	Microsatellite Instability-High
MTD	Maximum Tolerated Dose
NAb	Neutralising Antibody
NCCN	National Comprehensive Cancer Network
NIMP	Non-Investigational Medicinal Product
ORR	Objective Response Rate
OS	Overall Survival
PBMC	Peripheral Blood Mononuclear Cell
PD	Progressive Disease / Pharmacodynamics
PFS	Progression-Free Survival
PK	Pharmacokinetics
PO	Per Os (Oral)
PR	Partial Response
PRO	Patient-Reported Outcome
PS	Performance Status
PT	Preferred Term (MedDRA)
Q3W	Every 3 Weeks
QoL	Quality of Life
QLQ	Quality of Life Questionnaire
RECIST	Response Evaluation Criteria in Solid Tumors
RP2D	Recommended Phase II Dose
SAE	Serious Adverse Event
SAP	Statistical Analysis Plan
SAS	Safety Analysis Set

SOC	System Organ Class
STO22	Stomach Cancer Module 22 (EORTC)
TMB	Tumour Mutational Burden
ULN	Upper Limit of Normal
WOCBP	Women of Childbearing Potential

14. APPENDIX: REFERENCES

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END OF CLINICAL STUDY PROTOCOL — [REDACTED] — VERSION 1.0

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